

HUDSON VALLEY RADIOLOGY ASSOCIATES

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Exam requested by:
DAVID ANDERSON MD
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NEW YORK NY [REDACTED]

Patient: [REDACTED]
Date of Birth: [REDACTED]
Phone: (704) 349-0802
[REDACTED]
Date of Exam: [REDACTED]

EXAM: MRI BILATERAL BREASTS WITHOUT AND WITH CONTRAST

HISTORY: 46-year-old female recently diagnosed with left breast ductal carcinoma in situ.

COMPARISON: Screening mammogram from [REDACTED] diagnostic mammogram and breast ultrasound from [REDACTED] stereotactic core biopsy from [REDACTED] Screening mammogram from 2021, 2022, 2023.

TECHNIQUE: Images were acquired on a GE HDx 3T MRI with a dedicated breast array coil. Sagittal fat suppressed T2 weighted images were acquired (TR5167, TE99, FA90, 256 x 256 matrix, 4 mm slice thickness). Sagittal FSE T1 weighted images were acquired (TR500, TE18, FA90, 512 x 512, 5 mm slice thickness). Bilateral sagittal T1 weighted images were acquired (TR433, TE9, 4 mm slice thickness). Sagittal fat suppressed Vibrant gradient echo images were acquired through both breasts both before and after dynamic intravenous administration of gadolinium (TR7, TE2, TI6, FA10, Matrix 256 x 256, 2.8 mm slice thickness). These images were acquired dynamically out to 8 minutes. After the dynamic sagittal imaging, high resolution axial Vibrant images (TR6, TE2, TI18, FA10, Matrix 512 x 512, 1.4 mm slice thickness) were acquired. Images were reviewed utilizing breast MR software which includes subtraction post processing and enhancement curve analysis. Computer assisted diagnosis (CAD) was used. 20 ml of Clariscan was injected from a 20 ml single use vial.

FINDINGS:

Right breast: Background parenchymal enhancement is moderate. No [REDACTED] or morphologically abnormal right axillary lymph nodes.

Left breast: Background parenchymal enhancement is moderate. Susceptibility artifact is present in the medial mid left breast, likely corresponds to the stereotactic core biopsy labeled medial. There is no [REDACTED] the biopsy clip.

No [REDACTED] correspond to the second stereotactic core biopsy site.

A circumscribed enhancing mass in the upper outer mid to posterior left breast at approximately 2:30 o'clock 11 cm from the nipple measures 2.5 x 0.9 cm, not evident on the prior studies. Further evaluation with MRI guided core biopsy is recommended.

No [REDACTED]

IMPRESSION:

Confidential

Tel: [REDACTED] - Fax: [REDACTED]

ACCREDITATIONS .AMERICAN INSTITUTE OF ULTRASOUND IN MEDICINE.FETAL MEDICINE FOUNDATION.ACR AND MQSA FOR MAMMOGRAPHY

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1. 2.5 x 0.9 cm enhancing mass in the left breast at 2:30 o'clock 11 cm from the nipple. Further evaluation with MRI guided core biopsy is recommended.

2. No [REDACTED] to the biopsy-proven DCIS.

3. No [REDACTED]

FOLLOW-UP: Biopsy should be considered.

ASSESSMENT: BI-RADS Category 4: Suspicious findings.

Thank you for the opportunity to participate in the care of this patient.

DARIA SETLIK MD Bd Cert Radiologist - [REDACTED]
Physician [REDACTED]

FDA Site Name: [REDACTED]
505 RT 208 STES 14 & 18, MONROE, NY [REDACTED]

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